



Armada Kalamunda Group

Communication Disorder Management for Emergency Department Staff Guideline

1. Purpose

The purpose of this procedure is to assist in the diagnosis of medical conditions that present clinically as communication impairments, and to ensure patients with new communication impairments are appropriately assessed, diagnosed and managed.

2. Applicability

This guideline applies to all staff working in the Emergency Department (ED), including medical, nursing and allied health team members.

A variety of diagnoses can cause a person to experience changes to their communication or communication difficulties (i.e. stroke, Parkinson's disease, dementia). Speech Pathology (SP) don't have a consistent presence in ED, however are available to assess patients when required and rely on the ED allied health, medical and nursing staff to identify patients presenting with any communication impairments as a result of an acute or chronic neurological event and liaise with the AHS Speech Pathologists for assessment and/or management.

2.1 Referral

	Monday to Friday 8.30am - 3.30pm	After hours/ weekends:
Language disturbance Motor Speech Disorder Voice disorder/pharyngeal and laryngeal pathologies	Contact Speech Pathology for ED consultation (Pg 602/159/219) or call extension 2219/2395	- Is the person able to communicate their basic needs? If not – consider admission and refer to Speech Pathology for a consultation at the next available time (i.e. Monday or the following morning). - Provide the person with communication aid from ward communication kit - If discharged, refer to Speech Pathology for an outpatient appointment via WA Health e-referrals.

3. Roles and responsibilities

Role	Responsibility
ED Allied Health Team, Medical Team, Nursing Staff	Identify patients with communication needs and refer to Speech Pathology

Before referencing this guideline document please ensure that you have the latest version from the [Policy Library](#) information hub page.

Name

4. Legislative context

The following documents are required to give effect to this policy [i.e. the documents included are mandatory]:

- [WA Health Consent to Treatment Policy](#)
- [East Metropolitan Health Service \(EMHS\) Consent Policy](#)

5. Supporting information

The following documents support the implementation of this policy:

- [Speech Pathology - Inpatient Management Procedure](#)
- [Speech Pathology: Outpatient Services and Administration Tasks Procedure](#)

6. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 5 – Action 5.11: Clinicians comprehensively assess the conditions and risks identified through the screening process
- Standard 6 – Action 6.03: Clinicians use organisational processes from the Partnering with Consumers Standard to effectively communicate with patients, carers and families during high-risk situations to:
 - a. Actively involve patients in their own care
 - b. Meet the patient's information needs
 - c. Share decision-making

7. References

Australian Stroke Coalition. (2020). *Chapter 5: Rehabilitation*. In Australian and New Zealand living clinical guidelines for stroke management. Retrieved from [Australian and New Zealand Living Clinical Guidelines for Stroke Management - Chapter 5 of 8: Rehabilitation \(magicapp.org\)](#)

8. Glossary

The following definitions are relevant to this policy.

Term	Definition
Language disturbance	• Difficulty or inability to express themselves e.g.; word finding difficulty, putting words into fluent sentence • Difficulty understanding spoken language e.g.: following commands, answering biographical questions • Difficulty understanding written material - ensure reading glasses available and clarify person's pre-admission reading competence if possible • Difficulty writing – ensure reading glasses and appropriate writing implements are available

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Term	Definition
Motor speech disorder	• Difficulty with forming the sounds in speech (e.g. slurred, sounds different) • Difficulty with speech volume (e.g. very quiet speech) • Changes to the rate of speech (e.g. very slow or very rapid speech)
Voice disorder/pharyngeal and laryngeal pathologies	• Voice sounding different/abnormal (rough, weak or absent voice) • Pain when talking

9. Document owner

Coordinator Speech Pathology

Enquiries relating to this guideline may be directed to:

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10. Review

This guideline will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V3	15/11/2024	15/11/2027	Document changed format from a procedure to a 'guideline' Title: Changed the title to "Communication Disorder Management for ED staff guideline" as opposed to just for allied health staff (as it applies to medical, AH, nursing etc) 3. Guideline: New information added re: communication impairments 3.1 Referral: Removed EMR64.1 as referral option. Instead, included WA Health e-referrals as a referral mechanism.

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11. Authorisation

This guideline has been authorised and issued by the Director Allied Health and Ambulatory Care.

Authorised by	Danielle Kilmurray – Director Allied Health and Ambulatory Care
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